



Alkhidmat Foundation Pakistan

Summer Wehip Program 2026

Week 3 Report

Department: Volunteer Management

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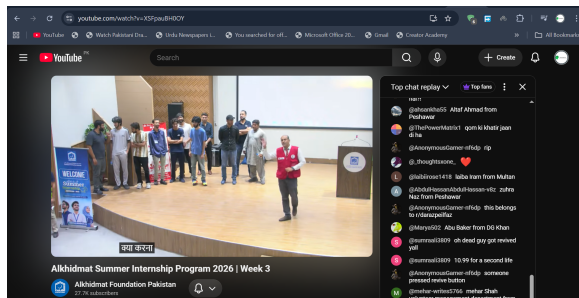
Submission Date: July 29, 2026

This report is submitted in fulfilment of the Week 3 deliverable of the Alkhidmat Summer Wehip Program 2026, covering Day 1 and Day 2 disaster preparedness and first aid training.

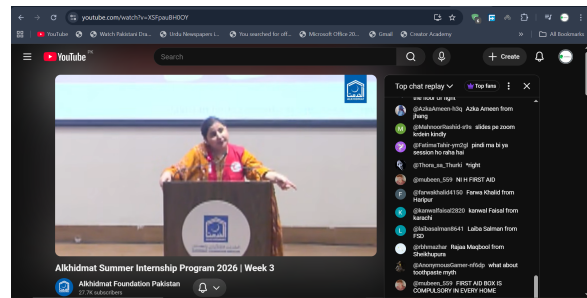
1 Day 1: First aid, Basic Life Support, and disaster preparedness

Conducted by the Pakistan Red Crescent Society, represented by Assistant Director Warda Mahmood, Zeeshan Shafique, and Sarah Nadeem, for the Alkhidmat Summer Wehip Program 2026.

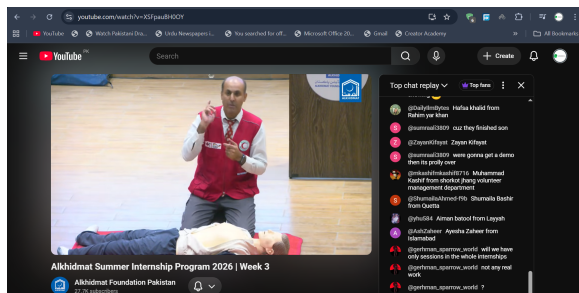
1.1 Proof of participation



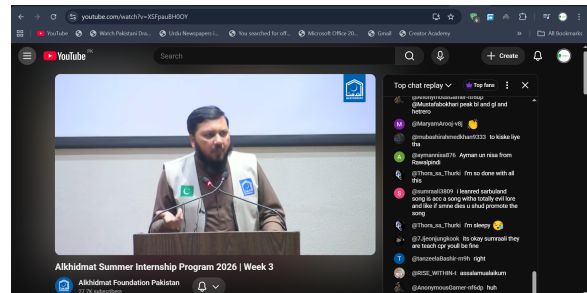
(a) Screenshot 1



(b) Screenshot 2



(c) Screenshot 3



(d) Screenshot 4

Figure 1: Proof of participation, Week 3 Day 1.

1.2 Key learnings

Core principles of first aid

First aid is immediate assistance given to a casualty, after calling for help, until professional medical care or an ambulance arrives. A first aider is expected to stay calm and confident, check the scene, call for help, take command, and apply common sense: the six Cs.

Emergency assessment: the DRABC framework

Cardiopulmonary resuscitation (CPR)

CPR is performed only when a person is unconscious and not breathing, for example after cardiac arrest, electric shock, drowning, or choking. The technique places the palm on the lower sternum with fingers locked and arms straight, delivering around 100 compressions per minute (range 90 to 120) using upper-body weight, combined as 30 compressions to 2 or 3 rescue breaths.

CPR is stopped only if the person revives, professional medical help arrives, the rescuer becomes physically exhausted, or a doctor declares death.

Step	Action
D – Danger	Remove any danger to yourself or the casualty before acting.
R – Response	Check consciousness by clapping, calling near the ears (about 6 inches away), or tapping the collarbone.
A – Airway	Keep the airway open with a head-tilt, chin-lift; check for tongue rollback or blockages.
B – Breathing	Look, listen, and feel for ten seconds. Normal adult breathing is 12 to 20 breaths per minute.
C – Call for help / CPR	Call emergency services and begin CPR if the person is not breathing.

Age group	Depth	Technique
Adults	About 2 in / 5 cm	Two hands
Children	About 1.5 in	One hand
Infants (0–6 months)	Specialized, gentle	Two fingers, careful positioning

Recovery position and common myths

An unconscious person who is breathing is laid on their side in the recovery position, which keeps the airway open and prevents choking on fluids or vomit. Two myths were specifically corrected: water should never be forced down the throat of an unconscious person, since this risks choking, and raw household items such as onions, potatoes, or tomatoes should never be applied to burns; a burn should instead be covered loosely with a clean cotton dressing.

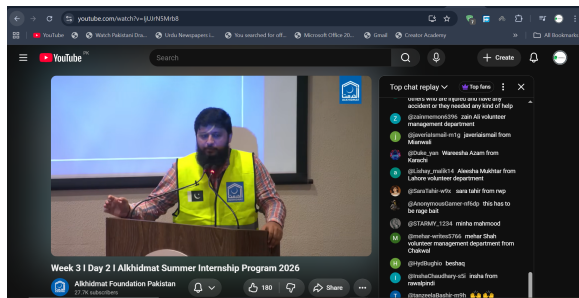
1.3 Outcomes

We gained practical awareness for managing emergencies including drowning, severe bleeding, electric shock, and burns. A cohort of 25 to 30 nominated We was selected for intensive, hands-on Basic Life Support training with the Pakistan Red Crescent Society. The session was broadcast live to over 25,000 virtual We across Pakistan.

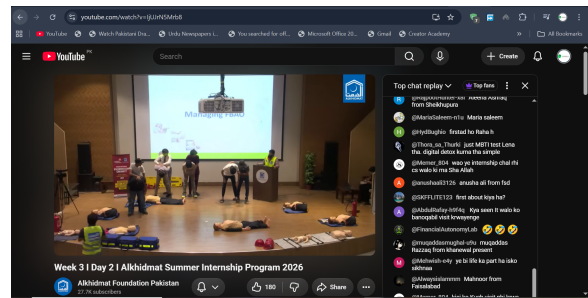
2 Day 2: Practical disaster management training

Conducted by Mohsin Naseer, Senior Program Executive of the Disaster Management Department at Alkhidmat Foundation Pakistan, with addresses by guest speakers Dr. Syed Ilahi, former Federal Health Minister and former Chairman of the Pakistan Red Crescent, and Syed Waqas Anjum Jafri, Secretary General of Alkhidmat Foundation Pakistan. The session covered basic first aid, CPR, firefighting, and water rescue.

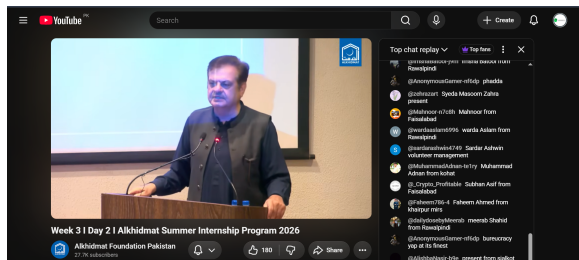
2.1 Proof of participation



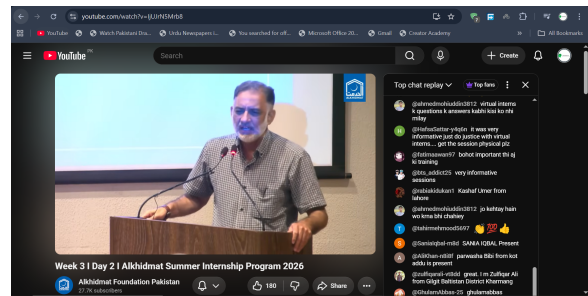
(a) Screenshot 1



(b) Screenshot 2



(c) Screenshot 3



(d) Screenshot 4

Figure 2: Proof of participation, Week 3 Day 2.

2.2 Key learnings

Emergency assessment and CPR: the Dr. CAP framework

This session updated the traditional assessment sequence to Dr. CAP: danger, response, chest compressions, airway, and proper positioning. Danger and response are checked first, by scanning for hazards and testing responsiveness through tapping the shoulders or clavicle and pinching, with consent obtained from bystanders where present. Chest compressions are delivered with the heel of the dominant hand at the centre of the chest, between the nipples, arms kept straight.

Age group	Depth	Technique
Adults	2 to 2.5 in	Both hands
Children (1–8 years)	1 to 1.5 in	One hand
Infants (under 1 year)	0.5 in	Two fingers

A cycle is 30 compressions followed by rescue breaths, or continuous compressions for 5 rounds or 10 continuous minutes. Clinical death, in the first 5 to 6 minutes after breathing or the heart stops, can be reversed with CPR; biological death, involving irreversible cellular decomposition, cannot.

Choking management: the Heimlich manoeuvre

Choking is recognised when a casualty clutches their neck with both hands. The rescuer stands behind the casualty, makes a fist positioned between the rib cage and the navel, and delivers inward and upward thrusts to force out the obstruction using pressure from the lungs.

Wound care, fractures, and improvised transport

Open wounds are cleaned with Pyodine or normal saline, covered with non-stick gauze rather than raw cotton placed directly on the wound, and wrapped with a bandage. Embedded objects such as knives, glass, or steel rods are never pulled out; instead they are secured in place with padding and a bandage to control bleeding around them. Fractures are splinted with rigid material, such as splints, hardboards, folded books, or sticks, before the casualty is moved, to prevent further bone displacement or tissue damage. For transport, the session covered improvised stretchers made from bamboo poles wrapped in shirts or blankets, along with three-person lifts, sheet pulls, chair carries, and firefighter carries.

Burns and chemical injuries

Thermal burns are flushed with plain running water; blisters are left intact, since they protect against infection. Liquid chemical burns are flushed immediately with large amounts of plain water. Powdered chemical spills are treated differently: the dry powder is brushed off with a cloth first, since pouring water on it directly can trigger a reaction with the chemical.

Nosebleeds and snakebites

For a nosebleed, the soft part of the nose is pinched just below the nasal bone for five seconds at a time, repeated three to four times, leaning forward with a damp towel on the forehead or neck; cold water over the head and tilting the head back are both avoided. For snakebites, around 85 percent of cases in Pakistan involve non-venomous species and leave a U-shaped bite mark. The wound is not cut, venom is not sucked out, and tight tourniquets are not applied for extended periods; the patient is kept calm and taken directly to hospital.

Firefighting and water rescue

Fire extinguishers are operated using the PASS method: pull the pin, aim at the base of the fire, squeeze the handle, and sweep side to side. Water is used on Class A fires (wood, cloth, paper); CO₂ or dry chemical powder on Class B, C, or E fires (flammable liquids, gases, live electrical equipment); and a wet chemical extinguisher or a damp cloth on kitchen oil fires. For water rescue, the session covered life jackets rated up to 120 kg, throw bags, throw guns, and outboard motorboats. After CPR, a drowning victim is placed in the right-side recovery position to help clear fluid from the stomach and lungs.

2.3 Outcomes

We practiced chest compressions on manikins, Heimlich thrusts, wound dressing, splinting, and improvised stretcher assembly, and worked through common first aid myths around burns, nosebleeds, electrical fires, and snakebites. Coordinators also clarified weekly reporting rules: each submission requires proof of participation, either onsite photos or virtual screenshots, along with key learnings and daily assignment responses merged into a single weekly submission. An extension was announced for Week 2 submissions, and details were shared for upcoming field site visits.

2.4 Assignment work

No separate assignment was issued for Day 2 beyond the community first aid awareness session assigned on Day 1.